



POLICY

Consent to Treatment

Scope (Staff):	All Staff
Scope (Area):	CAHS (CACH, CAMHS, PCH and Neonatology)

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with the [WA Health MP 0175/22 Consent to Treatment Policy](#)

This policy does not apply to the provision of treatment to patients who are adults.
Refer to the [WA Health Consent to Treatment Policy](#).

This policy does not apply to the release of information. Refer to: [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#).

While CAMHS is in scope for this policy, there is also a separate CAMHS Consent Guideline – [CAMHS Consent Guideline](#).

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Aim

To direct Child and Adolescent Health Service (CAHS) staff as to consent to treatment principles and processes in alignment with the [WA Health Consent to Treatment Policy](#).

Risk

- Provision of treatment without consent.
- Patient complaints which may manifest in legal consequences, such as claims for compensation and criminal liability.

Definitions

Capacity: The ability to make a treatment decision.

CCSA: *Children and Community Services Act 2004*.

Consent: Agreement to proposed treatment.

Explicit Consent: Consent that is clearly and unmistakably given.

Guardian: A person who is not a parent but who has parental responsibility for a child by virtue of:

- orders made by the Children's Court or the Family Court;
- a provision of the CCSA; or
- a deed or will left by a deceased parent (see [Testamentary Guardian](#)).

HTTA: *Human Tissue and Transplant Act 1982*.

Implied Consent: Consent that is implied when the actions of a person who can make the treatment decision indicate willingness to proceed with proposed treatment. For example, a mature minor holding out an arm for a blood test or a parent undressing a baby to allow them to be weighed by a Child Health Nurse.

Material Risks: Risks which, in the circumstances of the particular case, a reasonable person in the patient's position, if warned of the risk, would be likely to attach significance to, or if the health practitioner is or should reasonably be aware that a particular patient, if warned of the risk, would be likely to attach significance to.

Mature Minor: A child who is assessed to have capacity to make a treatment decision.

Medical record: In the context of CACH, includes the client health record.

Parent: A person with parental responsibility for a child.

Parental responsibility: includes responsibility for the:

- day-to-day care, welfare, and development of a child; and
- the long-term care, welfare, and development of a child.

Patient: Includes consumer and client. In appropriate contexts, includes a parent or guardian making a treatment decision on behalf of a patient.

Requisite Parental Responsibility: means the parental responsibility required to make the treatment decision as follows:

- minor or everyday treatment decisions may be authorised by a parent or guardian with responsibility for the day-to-day care, welfare and development of the child; and
- major or long-term treatment decisions may be authorised by a parent or guardian with responsibility for the long-term care, welfare and development of the child.

Special Medical Procedures: Medical procedures for which the approval of the Family Court or Supreme Court is required, such as:

- a major medical procedure that is not for the purpose of treating a bodily malfunction or disease; and
- in some circumstances, the provision of gender affirming care.

Testamentary Guardian: A person (or two persons jointly) appointed by a parent by deed or will, to be the guardian of a child following the parent's death.

Treatment: Any medical, surgical, dental treatment or other health care.

Key Principles

- Patients have a right to decide what happens to their own bodies. Subject to some limited exceptions, treatment cannot be provided without consent.
- In order to be valid, consent must:
 - be voluntary;
 - be informed by sufficient and meaningful information about the proposed treatment to enable an informed decision to be made;
 - be given by a person who has capacity to make the treatment decision;
 - cover the proposed treatment; and
 - be current in the context of the patient's present circumstances.
- Consent may be amended or withdrawn at any time.

Implied Consent

- Implied consent is sufficient in respect of treatments for which no significant risks to the patient are anticipated.
- Implied consent does not require a signed consent form.

- If there is doubt about whether there is implied consent, explicit consent must be sought.
- Attendance at a CAHS site does not, of itself, constitute implied consent to treatment.

Explicit Consent

- Explicit consent is required:
 - if it is unclear whether there is implied consent to treatment;
 - if the proposed treatment is more complex and/or involves Material Risks to the patient;
 - within PCHN and CAMHS, for the treatments set out in [Appendix A](#); and
 - within CACH, for the treatments set out in [Appendix B](#).

Process for Seeking Explicit Consent

1. Determine which health practitioner is responsible for seeking consent to treatment

- Within a clinical team a number of health practitioners may be involved in providing information to the patient that is relevant to their treatment decision.
- Consent to treatment is to be sought by a health practitioner with sufficient clinical knowledge of the proposed treatment.
- For treatments within PCHN and CAMHS that are performed by medical practitioners or require explicit consent, consent to treatment must be sought by a medical practitioner or a nurse practitioner.
- The health practitioner responsible for providing the treatment must be satisfied that a valid consent has been obtained and documented prior to providing the proposed treatment.

2. Assess who can make the treatment decision

- It is assumed that children do not have capacity to make treatment decisions.
- Despite this, children should be invited to:
 - participate in the decision-making process about proposed treatment commensurate with their level of development; and
 - provide assent to care whenever it is reasonable and appropriate in the circumstances.

Mature Minor

- A child assessed to be a mature minor for the purpose of a treatment decision may make that treatment decision for themselves. This includes a decision to refuse treatment.
- If a child is a mature minor for the purpose of a treatment decision, a parent or guardian may not make that treatment decision on behalf of the child.
- A child may be a mature minor for the purpose of only some or all treatment decisions.

Parent

- Generally, the consent of one parent is sufficient to authorise treatment in respect of a child who is not a mature minor.
- The parent is responsible for consulting with any other parent about the treatment decision as appropriate.
- Consent provided by one parent may not be sufficient if parental responsibility has been altered by a Court order. Consideration of the most recent Court orders will be required to ascertain which parent/s have the Requisite Parental Responsibility.
- The parent is responsible for bringing to the health practitioner's attention any Court orders that affect parental responsibility. Where it has not been possible to obtain a copy of the Court order from the parent, these can be requested through CAHS Legal Services.
- Separation, divorce, and restraining orders do not of themselves affect parental responsibility.

Guardians

- A guardian may make a treatment decision if they have the Requisite Parental Responsibility because of:
 - orders made by the Family Court; or
 - a protection order (special guardianship) made by the Children's Court.
- A guardian seeking to make a treatment order should provide to the health practitioner with a copy of the Court orders that authorise them to do so.
- A Testamentary Guardian may make a treatment decision if the parent who appointed them by deed or will was, at the time that they died, the only person with parental responsibility for the child.
- The CEO of the Department of Communities (Communities) may make treatment decisions in respect of some children in the CEOs care. Whether the CEO holds the Requisite Parental Responsibility in respect of a specific child will depend on the CCSA and the Children's Court order to which the child is subject (if applicable).
 - Refer to [Appendix C](#) for further guidance.

- To seek a treatment decision from the CEO of Communities, the health practitioner should contact the caseworker or Crisis Care (after hours).
- Foster carers, who provide care to some children in the CEO's care, are not authorised to make treatment decisions on behalf of the children placed in their care.
- Informal guardians, who care for a child in the absence of Court orders, are not authorised to make treatment decisions.

3. Assess the capacity of the person making the treatment decision

- Capacity must always be assessed in the context of the specific treatment decision to be made.
- While in many cases the assessment will be made by the treating clinician, in complex situations (such as the refusal of life-saving treatment by a mature minor), assessment by a psychiatrist may be appropriate.
- Assessment of capacity must be documented in the medical record.

Mature Minor

- Children are presumed not to have capacity to make treatment decisions. However, capacity will generally increase with age.
- Children will have capacity to make a treatment decision on their own behalf when they achieve sufficient understanding and intelligence to enable them to understand fully what is proposed, taking into account the significance of the treatment decision.
- Capacity to consent for one treatment does not automatically confer capacity to consent for subsequent treatments.
- While there is no set age at which a child achieves capacity, the following provides a general guide:

Level of Maturity & Understanding	Recommendation for Obtaining Consent
Immature and insufficient understanding (<i>may be 13 and under</i>)	Consent from a parent or guardian
Intermediate understanding (<i>may be 14 or 15</i>)	Consent from the child may be sufficient in some cases
Mature understanding (<i>may be 16 or 17</i>)	Consent from the child will be sufficient in most cases

- For further information about the assessment of capacity, refer to section 4.2 of [Working with Youth: A legal resource for community-based health professionals](#).

Parent or Guardian

- Adults are assumed to have capacity to make treatment decisions.
 - For parents who are children, refer to section above “Mature Minors”.
- If there are particular concerns arising as to the capacity of a parent or guardian to make a treatment decision, such as in the case of intoxication or other decision-making impairment, an assessment of capacity should be made, if possible.
- An adult has capacity to make a treatment decision if they can:
 - understand the nature, consequences and risks of the proposed treatment;
 - retain the information as necessary to make the decision;
 - use and weigh the information in the decision-making process; and
 - communicate their decision in some way.

4. Communication regarding proposed treatment to support informed decision-making

- The health practitioner must inform the person making the treatment decision of the following:
 - the nature of the proposed treatment, including likely outcome and potential benefits;
 - the general and specific Material Risks that are inherent in the proposed treatment;
 - any alternatives to the proposed treatment and the option to refuse treatment; and
 - the likelihood that the proposed treatment may be unsuccessful.
- Information must be appropriate in terms of the person’s health literacy, language, and culture.
- Where the person making the treatment decision has low literacy levels or vision or hearing impairment, the health practitioner must employ appropriate communication methods or enlist support to assist in communicating with the person.
- Where the person making the treatment decision requires an interpreter, an accredited interpreter must be engaged. Family members and friends should not be used to interpret information about proposed treatment.
 - Refer to [Language Services](#) (CAHS Policy Manual).
- Methods used to facilitate communication must be documented in the medical record. If possible, the interpreter should sign the declaration within the consent form.

- Aboriginal and Torres Strait Islander patients and families must be offered the option to speak to an Aboriginal Liaison Officer (ALO) if available, or if not available, to another Aboriginal staff member, and should be offered an interpreter if required.
- Where available, written information about the proposed treatment should be provided to the person making the treatment decision. However, written information is not a substitute for a meaningful discussion tailored to the person's needs.
- Where the proposed treatment includes anaesthesia, the person making the treatment decision must be provided with the [Anaesthetic Information for Parents brochure](#) and given a chance to have any questions answered.

5. Verify the understanding of the person making the treatment decision and address their queries

- The health practitioner must verify that the person making the treatment decision:
 - understands the effect of the treatment decision;
 - understands that a choice can be made;
 - has had sufficient time in the circumstances to consider and clarify the information presented;
 - can communicate key information about the proposed treatment and their decision back to the health practitioner; and
 - has had all their questions answered.

6. Seek the treatment decision

- The person making the treatment decision may consent or refuse consent to proposed treatment.
- Except as otherwise set out in this policy, treatment must not be provided without consent being given and documented using the appropriate consent form, or in the medical record.
- Consent may be amended or withdrawn at any time.
- Withdrawal of consent must be documented in the medical record, including the date and the person that withdrew the consent.
- Consent must be reviewed in response to changed circumstances. For example, if it has been more than 12 months since consent was provided or there have been changes in the patient's circumstances, clinical condition, available treatments and any associated material risks.

7. Courses of treatment

- Consent may be given to a course of treatment.

- Consent to a course of treatment must be documented in the medical record including all elements of the course of treatment, any associated Material Risks, alternative treatments available, and consequences of withdrawal at a future date.

8. Document the treatment decision

- Consent (and refusal of consent) may be given orally or in writing. The health practitioner must document oral consent (and refusal of consent) in the medical record.
- Consent (and refusal of consent) given in the context of a telehealth appointment is to be documented by the health practitioner on the appropriate consent form or the medical record. Where possible, the decision is to be confirmed in writing by the person making the treatment decision prior to treatment commencing.
- Relevant consent forms must be used.
 - For PCHN refer to [Appendix A](#).
 - For CACH refer to [Appendix B](#) and the [CACH Forms](#) page.
 - For CAMHS refer to the [Consent Guideline](#) (CAMHS Policy Manual).

9. Absence of Explicit Written Consent

- From time-to-time is it not possible to obtain explicit written consent to proposed treatment in accordance with this policy.
- All reasonably practicable steps should be taken to seek explicit written consent to proposed treatment, with these steps documented in the medical record. At a minimum these steps should include, if possible in the circumstances:
 - more than one attempt to contact the parent or guardian by telephone or text message; and
 - referral to the applicable social worker or after-hours social worker (for PCHN only).
- Escalate to the Divisional Co-Director or Executive Director Medical Services (during hours) or to the PCH Executive on-call (after-hours) if:
 - explicit written consent cannot be obtained despite all reasonably practicable steps being taken; and
 - the patient is at significant risk of irreparable harm if they don't receive the proposed treatment.
- The Divisional Co-Director, Executive Director Medical Services or Executive on-call will consider whether proposed treatment can proceed based on explicit consent that is provided orally.

10. Absence of Consent

- From time-to-time is it not possible to obtain consent to proposed treatment.
- The steps to be taken in the event of an absence of consent depend on the nature of the proposed treatment.
- **Note that this section does not apply if consent has been refused.**
 - See below [Conflicts about Proposed Treatment](#).

Emergency Treatment

- Emergency treatment may be authorised:
 - under the doctrine of necessity; or
 - by order of the Family Court or Supreme Court.
- Under the doctrine of necessity, treatment may be authorised if there is an overriding necessity to take action to protect the patient from a real and immediate danger of significant harm. It does not apply if the treatment is merely suitable, convenient or for the patient's benefit.
 - **Note that the doctrine of necessity does not apply if consent has been refused.**
- Any decision to provide emergency treatment under the doctrine of necessity must be made jointly by the treating Consultant and either the Divisional Co-Director or Executive Director Medical Services (during hours) or the Executive on-call (after hours).
- A decision to provide treatment under the doctrine of necessity must be documented in the medical record including:
 - the patient's clinical condition;
 - the clinical urgency of the treatment and the consequence of not providing the treatment, and;
 - efforts taken or not taken to obtain consent to treatment, and the reasons for this.
- If there is doubt as to the application of the doctrine of necessity in the circumstances, the Divisional Co-Director or Executive Director Medical Services (during hours) or the Executive on-call (after hours) will consider whether an emergency application should be made to the Family Court or Supreme Court to authorise the proposed treatment.
- If possible in the circumstances, legal advice should be sought prior to making any application to the Family Court or Supreme Court.

Urgent Treatment

- The treating Consultant is to escalate to the Divisional Co-Director or Executive Director Medical Services (during hours) or to the Executive on-call (after hours) if:

- consent cannot be obtained despite all reasonably practicable steps being taken; and
- the patient is at significant risk of irreparable harm if they don't receive the proposed treatment.
- The Divisional Co-Director, Executive Director Medical Services or the Executive on-call will consider whether an emergency application should be made to the Family Court or Supreme Court to authorise the proposed treatment.
- If possible in the circumstances, legal advice should be sought prior to making any application to the Family Court or Supreme Court.
- Note that the *Guardianship and Administration Act 1990*, which provides a framework to authorise the provision of “urgent treatment” to adults, does not apply to children.

Elective Treatment

- Elective treatment cannot be provided in the absence of consent.

Conflicts about Proposed Treatment

- The following circumstances can arise from time-to-time:
 - the clinical team recommend treatment that is refused by a person who can make the treatment decision;
 - there is a difference of opinion about the proposed treatment amongst the persons who can make the treatment decision; or
 - there is a difference of opinion about the proposed treatment between a mature minor and their parent/s or guardian/s.
- In the event of any conflict, the clinical team must work with the family to support decision-making that is focussed on the patient's best interests.
- In cases involving conflict between those with parental responsibility for the patient, it may be appropriate for those persons to seek appropriate orders in the Family Court.
- In cases where a refusal to provide proposed treatment may constitute medical neglect, the clinical team must seek advice from the Child Protection Unit.
 - It may be possible in some circumstances to exercise power under section 40 of the CSSA to keep a child under 6 years of age in hospital without parental consent. Note that the exercise of this power does not authorise the provision of treatment. Consent to treatment must be sought in accordance with this policy.
 - Refer to [Child Protection Unit Protocols](#) (PCH Operational Manual)

- Where the conflict relates to proposed treatment, without which the patient may suffer significant irreparable harm, the clinical team must escalate the matter to the Divisional Co-Director or Executive Director Medical Services (during hours) or to the Executive on-call (after hours) to consider whether an application should be made to the Family Court or Supreme Court to authorise the proposed treatment.
 - [Appendix D](#) sets out the procedure that applies to emergency treatment.
- If possible in the circumstances, legal advice should be sought prior to making any application to the Family Court or Supreme Court.

Treatment Without Consent

- There are some limited circumstances in which treatment may be provided without consent.

Blood Transfusions

- Under section 21 of the HTTA a medical practitioner may perform a blood transfusion for a child if:
 - the parent or guardian fails or refuses to consent, or they cannot be found following searches and enquiries that are reasonably practicable in the circumstances; and
 - the medical practitioner has consulted another medical practitioner and they agree:
 - as to the condition from which the child is suffering;
 - that the blood transfusion is a reasonable and proper treatment for that condition; and
 - that without a blood transfusion the child is likely to die; and
 - the medical practitioner who performs the blood transfusion on the child:
 - has had previous experience in performing blood transfusions, and;
 - is, before commencing the transfusion, satisfied that the blood to be transfused is suitable for the child.
- A blood transfusion means the transfusion of human blood, any constituent of human blood or saline solution or other liquid, into a child and includes the exchange of the whole or any part of the blood of a child and all medical and surgical procedures necessary to perform the transfusion or exchange.
- All decisions, discussions and assessments regarding the blood transfusion must be documented in the medical records. This includes the rationale for proceeding in the absence of consent or contrary to the wishes of a mature minor, parent or guardian.

Emergency Psychiatric Treatment

- Emergency psychiatric treatment under the *Mental Health Act 2014* may be provided to a child without informed consent.
 - Refer to [CAMHS Consent Guideline](#) (CAMHS Policy Manual).

School Health Services

- At times there may be concerns about the health and wellbeing of a child and standard methods of communication and efforts to obtain consent fail to elicit a response from a parent or guardian.
- Pursuant to an authorisation made by the Chief Health Officer under section 337(1) of the *Health (Miscellaneous Provisions) Act 1911*, nurses employed in specified positions within CACH are authorised to examine medically and physically, as the nurse deems necessary, any child attending any school.
- Consultation with the applicable Clinical Nurse Manager must occur prior to examination.

Specific Consent Requirements

- In some circumstances, specific or additional consent requirements apply.

Donation of Blood

- Under section 19 of the HTTA, specific requirements apply to the removal of blood from a child for non-therapeutic purposes.
- A proposal to remove blood in these circumstances must be escalated to the Executive Director Medical Services.

Donation of Regenerative Tissue

- Under sections 12 and 13 of the HTTA, specific requirements apply to the removal of regenerative tissue from a child for the purpose of transplantation to the body of another living person.
- A proposal to remove regenerative tissue in these circumstances must be escalated to the Executive Director Medical Services.
 - Refer to [Organ and Tissue Donation](#) (PCH Operational Manual).

Special Medical Procedures

- Special Medical Procedures require approval from the Family Court or Supreme Court. As such, consent from a parent or guardian, or from a mature minor, is not adequate.
- A proposal to provide a Special Medical Procedure (excluding gender affirming care) must be escalated to the Executive Director Medical Services.

Withholding or Withdrawal of Life-Sustaining Treatment

- Life-sustaining treatment may be withheld or withdrawn if the provision of that treatment is futile or non-beneficial.
- What is futile or non-beneficial will depend on the circumstances and be informed by the clinical advice and consultation with the mature minor, parent or guardian.

In the event of a conflict about the withholding or withdrawal of treatment:

- treatment for the acute deterioration of a patient who has suffered brain death may be withheld; and
- life-sustaining treatment must not otherwise be withheld or withdrawn.
 - Refer to the [Conflicts about Proposed Treatment](#) section.

Monitoring Compliance

- Audits of consent processes and documentation may be conducted as required.
- For CACH, staff are to be orientated to consent processes through service orientation programs. Compliance with obtaining valid consent for service is captured through the Client Record Audit and monitored by the Clinical Governance Committee.

References and related external legislation, policies, and guidelines
Children and Community Services Act 2004 (WA Government)
Consent to Treatment (WA Health)
Consent to Treatment Procedure (WA Health)
Guardianship and Administration Act 1990 (WA Government)
Health (Miscellaneous Provisions) Act 1911 (WA Government)
Human Tissue and Transplant Act 1982 (WA Government)
Language Services Policy (WA Health)
Mental Health Act 2014 (WA Government)
Western Australian Language Services Policy 2020 (WA Government)
Related CAHS internal policies, procedures and guidelines
Blood Products Prescription and Informed Consent (CAHS Transfusion Medicine Manual)

Child Protection Unit Protocols (PCH Operational Manual)
Children in Care – Conducting an Assessment (CACH Clinical Nursing Procedure)
Confidentiality, Disclosure and Transmission of Health Information (CAHS Policy Manual)
Consent (CAMHS Policy Manual)
Immunisation – Childhood (CACH Clinical Nursing Procedure)
Language Services (CAHS Policy Manual)
Media (Clinical) Management (CACH Operational Policy Manual)
Medication Management in Education Support Schools (CACH Clinical Nursing Policy)
New Referral Management (CDS Policy Manual)
Off-label psychotropic medication prescribing (Child Development Service Policy Manual)
Organ and Tissue Donation (PCH Operational Manual)
School Aged Health Services (CACH Clinical Nursing Policy)
Serial Casting for Idiopathic Toe Walking (Child Development Service Policy Manual)
Student Health Care Plans (CACH Clinical Nursing Guideline)
Universal Contact School Entry Health Assessment (CACH Clinical Nursing Guideline)

Useful resources (including related forms)
CAHS Standard Operational Procedures for the Approval of Research
CAHS-CH Forms HealthPoint Intranet
CAHS-MR Forms HealthPoint Intranet
Medical Imaging Policy Manual for PCH Medical Imaging consent procedures
National Statement of Ethical Conduct in Human Research 2023
PCH Anaesthetic Information for Parents brochure

[Working with Youth: A legal resource for community-based health professionals](#)

This document can be made available in alternative formats on request.

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Appendix A: PCHN treatments for which explicit written consent is required and applicable consent forms

- Explicit consent must be obtained and documented on the relevant consent form before proceeding with any of the treatments identified in the following table:

Treatment	Consent form
Administration of general or regional anaesthesia	Documented by the anaesthetist in the anaesthetic record and/or the patient's medical record
Surgical, medical or radiology treatments which require general or regional anaesthesia	Consent to Treatment and Administration of Anaesthetics form (MR 840.03) or via the electronic/digital consent form
Administration of medications with known high-risk complications	Consent for Medication Use Form (MR 505.00 / MR 859.01) or via the electronic/digital consent form
Transfusion of blood or blood products as part of a surgical procedure	Consent to Treatment and Administration of Anaesthetics form (MR 840.03) or via the electronic/digital consent form
Transfusion of blood or blood products other than as part of a surgical procedure	Consent to Blood Products Form (MR.843.80) or via the electronic/digital consent form Refer to the Blood Products Prescription and Informed Consent protocol

- Where it is not possible to document consent utilising the required consent form or via the electronic/digital consent form, the following matters must, as a minimum, be documented in the patient's medical record:
 - the patient's full name
 - the proposed treatment (including whether anaesthesia is required, insertion of an implantable medical device)
 - date/s and key points of consent discussion(s), including Material Risks and benefits discussed of the proposed treatment, questions asked by the person making the treatment decision and health practitioner responses
 - details about the information provided to the person making the treatment decision and the person providing the information (e.g. procedure specific information sheets)

- whether or not language services were required and if present (physically or virtually)
- the nature of the review of the patient's condition
- confirmation of consent prior to treatment (if applicable)
- confirmation of decline of consent, or if previously consenting, withdraw of consent (if applicable)
- full name and signature(s) of the health practitioner(s) who determined that the consent process has occurred
- signature of the person making the treatment decision and date signed

Appendix B: CACH treatments for which explicit consent is required and applicable consent forms

- Explicit consent must be obtained and documented on the relevant consent form before proceeding with any of the treatments identified in the following table:

Who relevant for	Treatment	Consent form	Related policy
Child Development Service			
All	Child Development Services for clients in the care of the Chief Executive Officer (CEO) of the Department of Communities	CHS723 Consent for Child Development Services or via the electronic/digital consent form	New Referral Management guideline
Medical Services	Off-label psychotropic medication use	CHS336 Consent for Off-Label Medication Use	Off-label psychotropic medication prescribing guideline
CDS Physiotherapists	Serial casting services	CHS303 Consent for Serial Casting form	Serial Casting for Idiopathic Toe Walking guideline
Nursing			
School Health	School Entry Health Assessment (SEHA) Parent/guardian not present	SEHA CHS409-1 form	Universal contact SEHA guideline
Ear Health	Ear health screening Parent/guardian not present	Consent for ear health school screening form CHS719	Audiometry procedure Otoscopy procedure Tympanometry procedure
Immunisation	Primary Schedule Immunisation	Recorded in the client record	Immunisation Service policy

Who relevant for	Treatment	Consent form	Related policy
			Immunisation - Childhood
Immunisation	School based Immunisation program	Consent can be provided via <u>VaccinateWA (external site)</u> , also accessed via <u>School-based immunisation program internet site</u> or if a hard copy form is required, liaise with the school	School based immunisation program
Education support services – medication administration	Medication administration	Recorded in the signed health care plan	Medication Management In Education Support Schools policy Student health care plans guideline

Appendix C: Treatment decisions by the CEO of Communities

- Section 127 of the CSSA authorises the CEO of Communities to make treatment decisions for some children who are in the CEO's care.
- The extent of the CEO's authority to make a treatment decision depends on the CSSA and the Children's Court order to which the child is subject (if applicable).
- Treatment decisions for the **day-to-day care, welfare and development** of a child are those decisions that are minor or everyday decisions.
- Treatment decisions for the **long-term care, welfare and development** of a child are those decisions that are major or long-term decisions.

Type of order	Treatment decision that can be made
Provisional protection and care s. 35 and 37.	Subject to any interim order made by the Children's Court (if applicable): • the CEO can make treatment decisions for the day-to-day care, welfare and development of the child; and • parental responsibility remains with the parents for treatment decisions for the long-term care, welfare and development of the child.
Protection order (supervision) s. 47	No treatment decision can be made by the CEO. Parental responsibility remains with the parents, though this may be subject to conditions.
Protection order (time-limited) s. 54	The CEO can make treatment decisions for the day-to-day and long-term care, welfare and development of the child.

Type of order	Treatment decision that can be made
Protection order (until 18) s. 57	The CEO can make treatment decisions for the day-to-day and long-term care, welfare and development of the child.
Protection order (special guardianship) s. 60	No treatment decision can be made by the CEO. Parental responsibility is transferred to the special guardian who can make treatment decisions for the day-to-day and long-term care, welfare and development of the child.
Negotiated placement agreement (NPA) s. 75	Subject to the express terms of the NPA, no treatment decision can be made by the CEO. Parental responsibility remains with the parents.
Placement service s. 32	No treatment decision can be made by the CEO. Parental responsibility remains with the parents.

Appendix D: Procedure for seeking Supreme Court authorisation where there is a conflict about emergency treatment

- **This procedure applies if:**
 - there is a conflict about proposed treatment, including because:
 - proposed treatment has been refused by a person who can make the treatment decision;
 - there is a difference of opinion about the proposed treatment amongst the persons who can make the treatment decision;
 - there is a difference of opinion about the proposed treatment between a mature minor and their parent/s or guardian/s; and
 - the proposed treatment is emergency treatment without which the patient may suffer significant irreparable harm.

1. Concern or issue to be escalated to the Consultant in charge of the patient who will attend in person and work directly with the family to support informed decision-making that is focussed on the patient's best interests.
2. The Consultant in charge of the patient to contact the Divisional Co-Director or Executive Director Medical Services (during hours) or the Executive on-call (after hours) for advice and approval to seek Supreme Court authorisation.
3. If approved by the Divisional Co-Director, Executive Director Medical Services or the Executive on-call, the Consultant in charge of the patient to telephone the Supreme Court on **9421 5333** and be ready to advise the following:
 - a. The Consultant's name and position;
 - b. The location of the patient;
 - c. The patient's name and date of birth;
 - d. The names and contact details of the patient's parents or guardians;
 - e. The patient's medical condition;
 - f. The proposed treatment;
 - g. Risk/s if the patient does receive the proposed treatment;
 - h. Risk/s if the patient does not receive the proposed treatment;
 - i. Any alternative treatment options, if applicable; and
 - j. Views of the mature minor (if applicable), parents or guardians in respect of the proposed treatment.
4. If the proposed treatment **is authorised** by the Supreme Court the Consultant in charge of the patient is to:

- a. Facilitate the provision of the treatment;
- b. Document all the circumstances and decisions in the medical record; and
- c. Seek support from [CAHS Legal Services](#) as soon as practicable via the Divisional Co-Directors or Executive Director Medical Services to formalise the application in writing, by providing an affidavit in support of that application.